

Sickle-cell disease or thalassemia You are tested to see if you are a carrier of sickle-cell disease or thalassemia, genetic disorders that affect the oxygen-carrying ability of red blood cells. These are most common in people of African, Hispanic, or Mediterranean origins. If you're found to be a carrier, the baby's father will need to be tested; if he is a carrier, there is a chance that your baby could develop the disease.

Routine urine tests

Routine urine tests are no longer needed; however, there may be times when your doctor recommends checking your urine for protein. If present, this may indicate an infection or, less commonly, kidney disease. Around 15 percent of women have bacteria, but no signs of infection. If bacteria are found, you may need antibiotics to stop kidney infection, which is more common in pregnancy and can cause complications. If no infection is found, more tests will check your kidney function. Later in pregnancy, protein in the urine can be a sign of [preeclampsia](#).

Getting your results

If all is well, you will be given the blood test results at your next visit. If there are any concerns, you will be contacted to discuss the results.

EXTRA SCREENING

Some additional tests may be offered.

- **Cervical cultures** Chlamydia and gonorrhea can be symptomless, so get tested if you think you're at risk.
- **Hepatitis C** You may be offered screening if you are high risk.
- **Varicella** A test can confirm whether you have chickenpox (varicella) immunity. If you're not immune and are exposed to the infection, treatment can help prevent severe chickenpox in pregnancy.
- **Toxoplasmosis** Being infected in the past will protect you from infection in pregnancy.

PRENATAL APPOINTMENTS

If your pregnancy is low risk, you will have about 10 prenatal appointments for a first pregnancy and about seven for second and subsequent pregnancies. After the first appointment with your doctor, your next visit will be at about 16 weeks.

At each appointment, your doctor will perform routine checkups, including taking your blood pressure and checking your urine for protein, the presence of which could indicate an infection or, later in pregnancy, [preeclampsia](#). You will be weighed regularly even if there are no particular concerns.

Before 16 weeks, your doctor will start to listen to your baby's heartbeat. Before 25 weeks, she will measure your abdomen to monitor growth (see [Measuring your baby](#)).